Breast adjuvant treatment only · NCCP 00588h. These materials cover olaparib after treatment for high-risk early breast cancer. Other cancers use different eligibility, duration and follow-up; use the matching NCCP indication and your treating team's plan.

NCCP 00588 · VERSION 5B

Olaparib · adjuvant breast treatment

Take olaparib tablets morning and evening every day. There is no planned week off. Blood-review appointments are usually every four weeks during this one-year breast course.

Discussion aid: review with the patient and complete the approved local consent process.

Purpose and plan

☐ Exact indication, treatment aim and uncertainty explained

☐ Expected duration and review / switch points discussed

☐ Alternatives and the option not to proceed discussed

☐ Patient priorities, capacity and support needs explored

Practical safety

☐ Medicine, route, schedule and missed-dose instructions

☐ Menopause and ovarian-suppression requirements if relevant

☐ Pregnancy, contraception / fertility and breastfeeding

☐ Monitoring, symptom reporting and contact plan

Selected material risks

☐ Nausea, vomiting, reduced appetite and indigestion

☐ Fatigue and anaemia

☐ Low neutrophils or platelets, with infection or bleeding risk

☐ Diarrhoea or altered taste

☐ Uncommon but serious pneumonitis or drug-induced liver injury

☐ Rare serious marrow disorders, including MDS/AML

☐ Venous thrombosis or pulmonary embolism; risk differs by treatment setting

☐ Fetal harm; contraception and breastfeeding restrictions

This patient suite is only for breast adjuvant indication 00588h. Kidney/liver impairment, previous dose reductions and interacting medicines can change the dose. New blood abnormalities can occasionally signal a serious marrow problem and must be investigated. Olaparib can harm an unborn baby. Discuss effective contraception before treatment; EMA recommends two reliable complementary methods for women during treatment and for six months afterwards. Men and their partners need contraception during treatment and for three months afterwards. Do not breastfeed during treatment or for one month after the last dose.

Individual benefit, alternatives and concerns

Agreed checks and next review

Confirm the germline BRCA result and breast eligibility. Review blood count, kidney and liver tests, recovery from previous treatment, medicines and supplements, and pregnancy testing where relevant. Confirm the surgery/radiotherapy sequence and the planned start. NCCP specifies FBC, kidney and liver tests every four weeks for the first twelve months. Reviews also assess nausea, appetite, energy, bruising, infections and breathing symptoms. Extra tests or an interruption may be needed if results or symptoms change.

Clinician / date: _____ Patient discussion documented: _____

Consent is an ongoing conversation and may be reconsidered. This aid is not treatment authorisation.